

The health certificate in international movement of dogs and cats: clinical examination, health traceability and document validity

Jessica Ysabel Camacho Garcia, M.V.Z., CMVP 12434

Zoovet Travel, Trujillo, Peru

ORCID: 0009-0002-6837-5311

Technical Series Vol. X - Zoovet Travel

DOI: 10.5281/zenodo.20209709

Publisher: Zoovet Travel - zoovettravel.com

Trujillo, La Libertad, Peru - 2026-05-15

Abstract

Objective. To describe the clinical-documentary nature of the veterinary health certificate in international movement of dogs and cats, its health basis, required examination components, and frequent causes of invalidity. **Method.** Technical review of international regulatory frameworks, pre-transport clinical examination protocols, and documentary traceability requirements of major destination countries. **Results.** The health certificate is a technical declaration under professional veterinary responsibility, not a guarantee of disease absence. Its validity depends on microchip-species-date concordance, the time window (generally 10 days), the type of authorized issuer, and official endorsement when required. Traceability errors are the most frequent cause of border rejection. **Conclusion.** The distinction between a veterinary certificate and an officially endorsed certificate is decisive for export file planning according to the specific destination country.

Keywords: veterinary health certificate, clinical examination, health traceability, official endorsement, pet export, document validity, international movement

Review of the clinical and regulatory basis of the international veterinary health certificate, its role in health control of companion animal movement, and the distinction between clinical certificate and officially endorsed document.

Jessica Ysabel Camacho Garcia, DVM -- CMVP 12434 -- Zoovet Travel, Lima, Peru | Revision: February 2026

Scope declaration -- mandatory reading This article is a technical descriptive document. It is not legal or individualised veterinary advice. It does not replace the official regulations of any jurisdiction nor substitute for the assessment of the responsible veterinarian. Requirements regarding the health certificate vary by country, route, species, transport operator and airline. Regulations change frequently and without prior notice. Information on the internet, including this document, may not reflect the most recent amendments. Direct and up-to-date verification with the competent health authority of the country of destination and of all transit countries is mandatory before issuing any health certificate or initiating any animal export process. The issuance of the health certificate is the exclusive responsibility of the veterinarian authorised under the regulations in force at the time and place of issuance. This article does not interfere with that professional judgment.

Section 1 What a health certificate is: clinical act and health control instrument

1.1 Clinical-documentary nature

The health certificate is the written formalisation of a clinical examination performed for a specific purpose: to determine whether an animal presents, at the time of examination, clinical conditions compatible with international transport and with the sanitary requirements of the destination. It is not a diagnostic document nor a complete medical report: it is a circumscribed technical declaration, with defined scope and precise reference date.

Understanding this dual nature --clinical in origin, documentary in function-- is essential to correctly interpret both its possibilities and its limitations. The health certificate translates a biological event --the animal's state at a given moment-- into a standardised administrative format that can be read, interpreted and verified by persons and systems that were not present at the examination.

This translation inevitably introduces a simplification. An animal's clinical state is continuous and dynamic; the certificate captures it as a static snapshot. This limitation does not invalidate the instrument: it defines its scope. International health control operates on documents because it cannot operate on individuals in real time. The certificate is the best available approximator within that constraint.

1.2 Certificate vs. guarantee: a distinction that must not be lost

The word «certificate» frequently induces a mistaken interpretation: that the document certifies, in the sense of guaranteeing, that the animal is free of disease. This interpretation is not technically correct and, when it guides the decisions of owners or transport operators, generates expectations that the instrument cannot satisfy.

What the health certificate certifies is that an authorised veterinarian examined the animal on a specific date and did not find, in that examination, clinical findings that contraindicate transport under the described conditions. That is functionally distinct from guaranteeing that the animal is healthy in an absolute sense, that it is not a carrier of any infectious agent, or that it will not develop disease during or after travel.

The difference is not semantic. An animal may pass a clinical examination with favourable result and be in the incubation period of a disease that does not yet produce detectable signs. An animal may have a subclinical condition that is not relevant for transport but that manifests after travel. The international health control system is aware of these limitations and manages them with other instruments --vaccination, serological tests, quarantine-- precisely because the health certificate can only do what a clinical examination can do.

1.3 The issuing veterinarian: technical declaration under professional responsibility

The veterinarian's signature on a health certificate is not a formal validation of a completed form. It is a technical declaration under professional responsibility affirming that the examination was performed, that the recorded findings correspond to what was observed, and that the professional's clinical judgment supports the statements contained in the document.

This responsibility has practical implications in both directions. The veterinarian cannot sign a health certificate based on an examination not performed, on data reported by third parties, or on prior documentation without current examination. Nor can the veterinarian complete a certificate with statements that the examination does not support. In jurisdictions where the health certificate requires endorsement by a health authority, the initial issuer must be aware that the declaration will be reviewed and potentially validated or rejected by a third party with legal competence to do so.

Section 2 Health basis: why States require health certificates

2.1 Biosecurity and pathogen introduction control

International movement of companion animals is, from the perspective of veterinary epidemiology, a potential vector for introduction of infectious agents into animal and human populations that have not had prior contact with them. The history of international animal health documents cases of introduction of disease through legal and illegal movement of companion animals, which justifies the existence of control systems regardless of the individual risk of each specific animal.

The health certificate is one of the instruments of that control system. Its function is not primarily to protect the animal that carries it --although it may contribute to that-- but to protect the animal and human populations of the receiving territory. This veterinary public health perspective explains why States require documentation even for animals from countries with high vaccination prevalence and low incidence of relevant diseases: the risk

is not only the individual animal, but the chain of transmission that could be initiated if an asymptomatic carrier comes into contact with a naive receiving population.

2.2 Individual risk vs. population risk

The distinction between individual risk and population risk, already introduced in Article 9 of this series in the context of vaccination, is equally central to understanding the health certificate. A specific animal may have an extremely low individual risk of carrying or transmitting any relevant pathogen. But the health control system does not evaluate individual animals: it evaluates flows of animals and manages collective probabilities.

If a million animals cross a border per year and the system requires a health certificate, the objective is not to detect the few carriers among the million --that diagnostic capacity is beyond what a clinical examination can offer-- but to create a traceability mechanism that allows, when a health event occurs, reconstruction of the movements involved, identification of the origin of the problem and containment of its spread.

This retroactive traceability function is, in many cases, more important than the prospective screening function of the certificate. A certificate that cannot be traced back to the specific animal that carries it, to the veterinarian who issued it, and to the exact moment of examination, has lost its main utility as an instrument of population health control.

2.3 Why validity time windows exist

The health certificate has an issuance date and a validity window: a period after which the document is no longer accepted by destination authorities, regardless of the correctness of its content. This document expiration is frequently perceived as an arbitrary bureaucratic requirement, but it has basis in risk control logic.

A clinical examination performed thirty days ago describes the animal's state thirty days ago. An animal that was clinically healthy thirty days ago may have developed a disease in that interval, or may have been exposed to an infectious agent still in incubation period. The greater the interval between examination and travel, the less information the certificate provides about the animal's current state.

The time windows required by different destinations --which vary and must be verified with the competent authority of each jurisdiction in each specific case, as they are neither universal nor static-- reflect the balance between logistical practicality (the owner needs time to organise travel after the examination) and the document's informative utility (the clinical state at the time of examination must be reasonably close to the clinical state at the time of travel). That balance is a health policy decision, not a direct biological inference.

Section 3 Typical clinical components of the examination

Editorial note -- Section 3 The components described in this section reflect what regulatory practice and veterinary clinical literature describe as usual content of an examination oriented to issuance of a health certificate for international transport. They do not constitute a prescriptive protocol nor a clinical guideline. The selection of findings and procedures depends on the specific case, on destination requirements and is the exclusive decision of the responsible veterinarian.

3.1 Identification and document consistency

The first component of the examination oriented to certification is not strictly clinical: it is verification that the animal before the veterinarian is the animal described in the documents accompanying the file. This implies reading the microchip with a compatible reader and checking that the number obtained matches exactly with that registered in prior health documents. It is the step most frequently omitted in rushed examinations and the one that generates the error with greatest impact at the border.

3.2 General condition assessment

The general assessment of the animal includes evaluation of level of consciousness and response to the environment, body condition, apparent hydration status, and general attitude. These parameters, assessed observationally and through basic palpation, provide

information on the animal's systemic status and its foreseeable tolerance to transport stress.

In the specific context of the health certificate for international transport, what is relevant is not exhaustive diagnosis but detection of signs that contraindicate travel under the foreseen conditions: signs of active disease, systemic compromise, significant dehydration, or extreme body condition. Absence of those findings is what the certificate declares; not absence of all pathology.

3.3 Systems relevant for transport

Regulatory frameworks and certification-oriented clinical practice typically contemplate assessment of the systems most relevant for tolerance to air transport:

- Respiratory system (observational): resting respiratory rate and pattern, presence of dyspnoea, increased respiratory effort or evident secretions. In brachycephalic breeds, this component acquires particular relevance given the risk profile described in Article 8 of this series.
- Cardiovascular system (basic): heart rate and pulse quality in the context of examination. Findings suggesting cardiovascular decompensation relevant for transport.
- Apparent neurological status: level of consciousness, response to stimuli, ambulation capacity. Active neurological signs that compromise the animal's ability to tolerate confinement or that indicate active central pathology.
- Integument and visible ectoparasites: some destinations require declaration of absence of visible ectoparasites as part of the health certificate. Integument assessment may also reveal dermatological signs compatible with zoonotic conditions relevant for the destination.
- Signs compatible with active infectious disease: fever, generalised lymphadenopathy, active mucous secretions, or any finding compatible with ongoing infectious process are findings whose presence contraindicates certificate issuance at that moment.

3.4 What the examination cannot rule out

As important as what the examination evaluates is clarity about what it cannot rule out. A clinical examination of reasonable duration, without supplementary tests, cannot rule out subclinical infection, asymptomatic carrier states, organic pathology without external clinical manifestation, or diseases in incubation period. The health certificate that results from that examination cannot do so either. This limitation is inherent to the instrument and must be understood by all actors in the process: veterinarian, owner, transport operator and health authority.

Section 4 Time windows and validity: biology vs. administration

4.1 Basis of time windows

The validity time window of a health certificate --the period between its issuance and the moment of travel when it is still accepted-- is one of the most frequently misinterpreted aspects of the international transport documentary system. There is no single universal window: each destination, and in some cases each transport operator, establishes its own, which may vary between 24 hours and 10 days or more, depending on multiple factors.

What is universal is the underlying logic: an animal's clinical state is temporary. A certificate issued a week ago describes the animal's state a week ago. The time window is the period during which the regulatory system considers that there is sufficient temporal proximity between examination and travel for the document to remain informative about the animal's current state.

When the time window expires, the certificate does not become medically useless: it still describes what the veterinarian found at that time. What it loses is its administrative validity: the destination control system no longer accepts it as adequate representation of the animal's current state.

4.2 Logistical implications of short windows

The shortest time windows --those requiring that examination and certificate issuance be performed in the days immediately before travel-- have a direct logistical implication: the examination cannot be performed with sufficient advance notice for the owner to reorganise travel if findings are detected that contraindicate certificate issuance.

This is, from the perspective of planning international animal transport, one of the variables of highest operational risk. An animal that presents mild febrile process the day before the flight, or that has just undergone a veterinary procedure that the examiner considers relevant for transport, may find itself in a situation where the certificate cannot be issued and travel must be suspended or postponed with the logistical and economic consequences that entails.

Managing this risk does not consist of artificially extending time windows --that would compromise their control function-- but of planning with sufficient time margin for any medical contingency in the days before travel to be resolved without compromising the overall schedule.

4.3 Certificate windows vs. complete file windows

The health certificate has its own validity window, but it is not the only document in the health file with a reference date. rabies vaccination has a validity period. RNATT, when required, has a reference date. Antiparasitic treatments, when required, have a specific window in relation to the date of entry to the destination. The official export certificate itself, in systems that contemplate it, may have a circumscribed issuance window.

Temporal coherence of all these documents with each other --and of each with the travel date-- is one of the most complex verification elements of the export process. A file in which each document individually complies with its requirements but in which dates are not coherent with each other may be rejected at the border with the same consequence as if one of the documents were missing.

Section 5 Document consistency and traceability: where travels fail

If there is a common pattern in documentary problems that generate delays, rejections or unplanned quarantines in international movement of companion animals, it is internal inconsistency of the file: documents that are individually correct but that are not consistent with each other, or that do not correspond to the animal presented at the border. Traceability --the capacity to unequivocally link the individual animal with its entire documentary chain-- is the principle that the control system verifies systematically.

5.1 The microchip as the file's common thread

The 15-digit ISO microchip number is the identifier that must appear exactly and identically in each document of the health file: the health certificate, the vaccination certificate, the RNATT result when applicable, the European passport or official health certificate, and any additional document required by the destination or operator.

This redundancy is not unnecessary: it is the mechanism that enables cross-verification. If the microchip number varies between documents --even in a single digit-- file consistency is broken. The border control authority cannot assume that the discrepancy is a typographical error: it must assume that the documents could correspond to different animals. The documentary consequences of that ambiguity are, in most systems, detention of movement until the inconsistency is clarified.

5.2 Frequent technical errors that invalidate the certificate

Analysis of the most frequent documentary problems in international animal movement allows identification of a recurrent taxonomy of errors. None requires bad faith; most are the result of rushed processes or incorrect assumptions about destination regulations:

5.3 Verification as systematic practice, not exceptional

Systematic review of the complete file --verifying that each document is internally correct and that all are consistent with each other-- should not be an emergency step performed the day before travel. It is a practice that, incorporated into the preparation process with sufficient advance notice, allows identification and correction of most errors before they

have irreversible consequences.

The most useful practical rule is to read each document as if it were the first: without assuming that what appears in the other documents is consistent with this one. And to verify the microchip number in each of them, comparing it with that read directly on the animal, before considering the file closed.

Section 6 Critical distinction: veterinary certificate vs. officially endorsed certificate

6.1 Two types of documents, two levels of validity

In the international health control system for companion animal movement, two types of documents coexist that are frequently confused under the generic term «health certificate»:

- The veterinary certificate: issued by a veterinarian authorised in the private practice of the profession, on the basis of clinical examination and professional judgment. It is a medical document with individual professional responsibility. Its validity is that of the professional who signs it.
- The official certificate or endorsed document: is the result of a process in which the veterinary certificate, or an official version of it, is reviewed and validated by a competent health authority of the State. This endorsement transforms the document: it is no longer just the declaration of a private professional, it is an official document backed by the authority of the issuing State.

This distinction is not formalistic. It has direct implications for the value that the receiving State assigns to the document. A veterinary certificate without official endorsement may not be accepted by destinations that require the document to come from an official veterinary authority of the country of origin. The same clinical content, without the endorsement, may be insufficient to comply with import requirements.

Key distinction: veterinary certificate vs. officially endorsed document
Veterinary certificate: issued by veterinarian in private practice. Individual technical declaration. Validity = signing professional's judgment. Officially endorsed certificate: reviewed and

validated by competent health authority of the State. Official document backed by issuing authority. Requirement in numerous destinations.

6.2 Examples of systems with endorsement: frameworks verified in this series

Two regulatory systems analysed in detail in previous articles of this series illustrate the distinction clearly:

European Union system (for movement from third countries): Reglamento (UE) 576/2013 and its implementing regulations establish official health certificate models that must be issued by the official veterinarian of the country of origin. A private veterinarian's certificate is not sufficient: the document must correspond to an approved model and must be signed by a veterinarian with recognised official status. This system represents endorsement integrated into the document format itself.

United Kingdom Animal Health Certificate: the AHC required by DEFRA/APHA for entry of animals into Great Britain must be issued by an accredited Official Veterinarian (OV) authorised by the competent authority. A private veterinarian without that specific accreditation cannot issue a valid AHC, regardless of clinical competence. The document itself constitutes the endorsement, because its issuance is reserved to professionals authorised by the authority.

6.3 Implications for the export process

The existence of endorsement or official certification requirements has direct logistical implications that must be integrated into export process planning:

- The veterinarian who will perform the clinical examination must be specifically authorised to issue the type of document required by the destination, not only to practise veterinary medicine in general.
- Endorsement processes by health authorities may have processing times that are incompatible with very short validity time windows. Coordination between examination date, endorsement date and travel date must be planned with sufficient margin.

- The document model to be used must be verified on the official site of the destination authority before issuance, not after. An endorsement on a document model not accepted by the destination lacks validity.

For all destinations not explicitly analysed in this series, the correct question is not «do I need a health certificate?» --the answer is invariably yes-- but «what type of document is required, who can issue it and in what format?» The answer to that question can only come from the competent health authority of the destination.

Section 7 Transport operators: requirements parallel to the State framework

7.1 State regulation vs. operational policy: two superimposed layers

The State regulatory framework for international animal movement --regulations of countries of origin, transit and destination-- establishes the minimum requirements for legal import of the animal. However, air transport of animals adds a second layer of requirements: the transport operator's operational policy, which may be more restrictive than State regulations and is not subject to the same publication and updating processes as official regulations.

This superposition creates a situation in which an owner may have complied with all State regulatory requirements to move their animal and find, at the moment of embarkation, that the transport operator has additional conditions not included in their preparation. Ignorance of operator policy is, from the transport system's perspective, the sender's responsibility.

7.2 Additional documentation that operators may require

Air transport operators --airlines for animals in cabin or hold, pet relocation courier services, air cargo operators-- may require, independently of what destination regulations demand, additional documents aimed at managing their own operational risk:

- Fitness to fly declaration (fit to fly): a specific document, distinct from the general health certificate, in which the veterinarian declares that the animal is fit specifically for air transport under the foreseen conditions of the specific flight. This document may have its own format, its own time window and its own issuance requirements according to the operator.
- Operator's own forms: some airlines and specialised operators have their own forms that must be completed by the veterinarian, separately or in addition to the official health certificate.
- Restrictions by species, breed or condition: operators may prohibit or condition transport of certain breeds (mainly brachycephalic), age ranges (very young puppies, geriatric animals), or animals with certain medical conditions, regardless of whether destination regulations contemplate that restriction.
- Kennel requirements: specifications for dimensions, material, closure system and ventilation of the kennel are frequently more detailed in operator policies than in regulatory requirements. Kennel non-conformity may be cause for embarkation rejection.

7.3 Managing the gap between regulation and operational policy

The gap between what State regulations require and what the transport operator requires can be managed with a simple principle: always verify directly with the specific operator that will perform the transport, with sufficient time for any additional requirement to be fulfilled before the day of travel.

Operator policies are not published in the same way as official regulations. They may change without prior notice, may vary between routes of the same operator, and may have exceptions known only by contacting directly the live cargo or special services area of the operator. Third-party information --including technical articles such as this one-- does not substitute for that direct verification.

Section 8 Limitations and professional responsibility

8.1 The certificate reduces risk but does not eliminate biological uncertainty

The complete documentary system that accompanies an animal in international movement --microchip, vaccinations, RNATT when applicable, antiparasitic treatments, health certificate, officially endorsed certificate-- is the best available system for managing the collective risk associated with movement of animals across borders. It is not a perfect system, nor can it be, because it operates on necessarily incomplete information regarding the real biological state of each individual.

Biological uncertainty is irreducible with current assessment instruments. An animal may pass all available documentary and clinical controls and yet be a carrier of an infectious agent that does not produce clinical signs, that is not covered by any of the required vaccines, or that is in too short an incubation period to have been detected in the examination. The control system assumes this limitation and manages it by reducing the probability that this occurs to levels acceptable for its public health objectives, not by eliminating it.

8.2 Changing regulations: the imperative of updated verification

Throughout this technical series, and in particular in the articles on vaccination certificates and health certificates, a warning has been repeated that is not rhetoric: regulations change frequently. This frequency is not uniform: some regulatory frameworks are stable for years, others are modified with each new epidemiological evidence, each outbreak of relevant disease, or each health policy change of the governments involved.

The veterinarian who works in international travel medicine for companion animals cannot rely on knowledge acquired at a given moment to make decisions at a later moment without verification. The temporal distance between acquisition of regulatory knowledge and its application may be sufficient for that knowledge to be incorrect. The only reasonable guarantee of documentary compliance is direct verification with the competent authority at the moment of issuance.

8.3 Scope of this article and its own limitations

This article describes principles, conceptual frameworks and error patterns. It does not provide country-by-country requirement lists because those lists expire and their inclusion here would generate a false sense of completeness and update that could be more dangerous than the absence of that information. The editorial criterion of this series has been consistent since the first article: rigour before exhaustiveness, and honesty about limits before completeness commitments that cannot be maintained.

Section 9 Final synthesis

The health certificate is the interface between clinical biology and international health control. It is the point where the state of a living organism --continuous, dynamic, partially opaque to external evaluation-- becomes a static, standardised and verifiable document that can accompany that organism through control systems designed to process information, not biological individuals.

The quality of that document, and with it its real utility as a health control instrument, depends on four conditions that must be fulfilled simultaneously:

- Real and documented clinical examination: an examination performed with the rigour that the context demands, with findings recorded faithfully, on an animal that is the same as that appearing in the file documents.
- Correct and consistent identification: the ISO microchip number verified on the animal and registered exactly and identically in each document of the file, from the first vaccination to the last document of the process.
- Compliance with regulations in force at the moment of issuance: correct format, adequate authorisation of the issuer, official endorsement when required, deadlines respected. Not compliance with what regulations required six months ago, but what they require at the exact moment the document is issued.

- Consistency of the complete documentary chain: each document in the file is individually correct and all are consistent with each other. Dates are compatible, animal descriptions are consistent, and the microchip number is the same in each and every one of them.

When these four conditions are fulfilled, the health certificate fulfils its function: to certify before the control system that the animal that carries it reached that point through a verifiable medical and documentary process. When one fails, the document loses that capacity, regardless of the correctness of the others.

That interdependence --not the complexity of any individual step-- is what makes the certification process in veterinary travel medicine a discipline that requires systematic attention, continuous verification and permanent updating. Competence in that space is not static: it is built and maintained.

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Figure 1. Health-certificate validity at destination — decision flowchart.

All five conditions are conjunctive (AND-logic). The exact temporal window and endorsement requirement depend on the destination and must be verified.

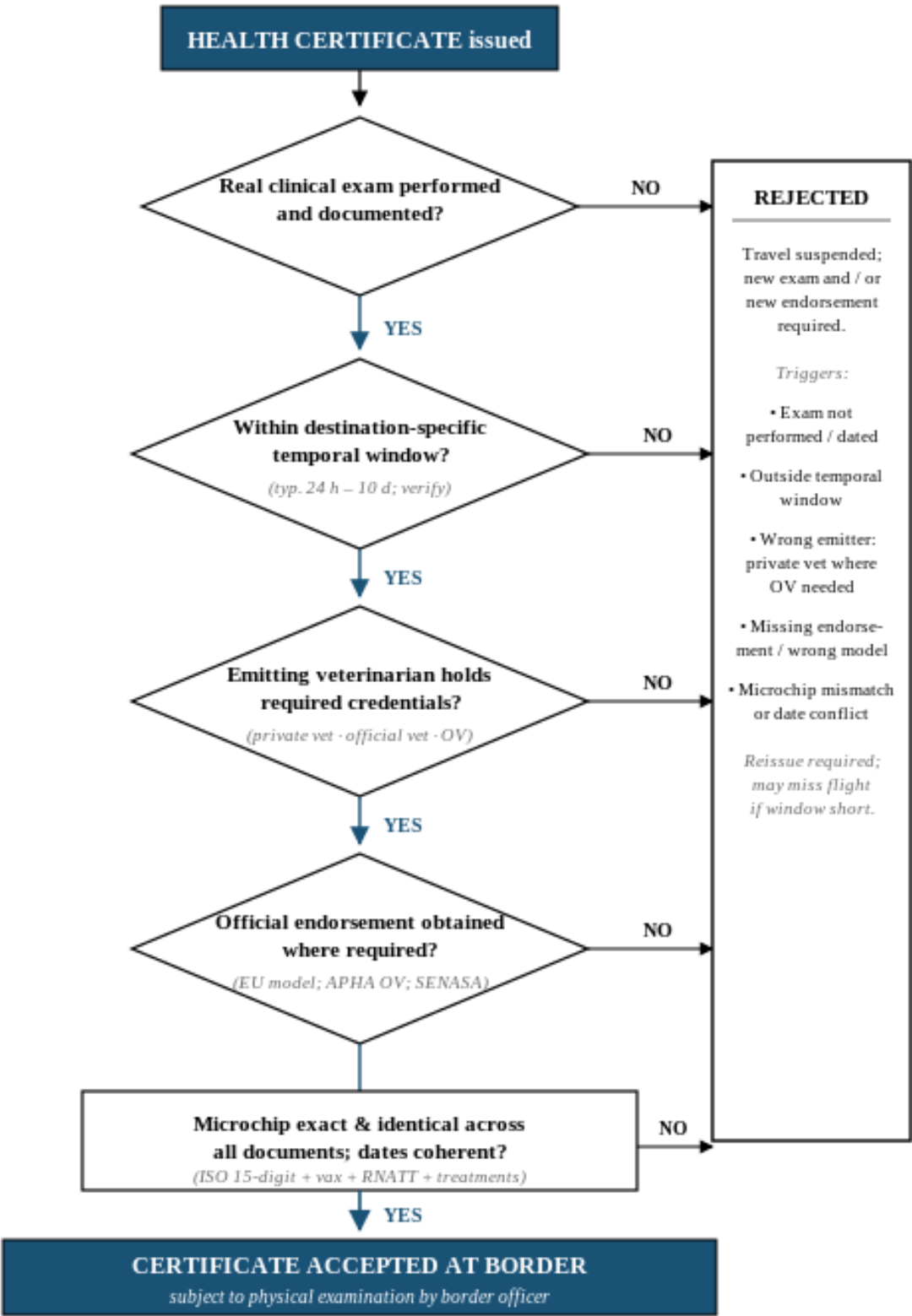


Figure 2. Private veterinary certificate vs. officially endorsed certificate — comparative matrix.

Synthesis of EU Reg. 576/2013, UK APHA AHC scheme, AU DAFF, NZ MPI. Direct verification with each authority is required before emission.

Attribute	Private veterinary certificate <i>individual professional responsibility</i>	Officially endorsed certificate <i>state-level authority document</i>
European Union Reg. (EU) 576/2013 Intra-EU + third country routes	Status: Insufficient for third-country entry Emitter: Any habilitated private vet Use: Pre-export clinical screening only <i>Border officer requires endorsed version</i>	Model: Impl. Reg. 577/2013 Annex IV Emitter: Official veterinarian (state) Window: ≤ 10 d before entry <i>In Peru: SENASA-accredited official vet</i>
United Kingdom APHA / DEFRA Pet Travel Animal Health Certificate (AHC) — post-Brexit	Status: Not accepted as AHC Emitter: Private vet — no OV status Use: Internal clinical documentation <i>AHC reserved to APHA-habilitated OVs</i>	Model: Animal Health Certificate Emitter: Official Veterinarian (OV) Window: ≤ 10 d before entry, single use <i>OV credential issued by APHA</i>
Australia DAFF Biosecurity Act Import permit + Health Certificate per group	Status: Not accepted alone Emitter: Private vet Use: Pre-RFFIT and pre-protocol <i>Requires official certification chain</i>	Model: Group-specific veterinary cert. Emitter: Government-approved vet Window: ≤ 5 d before export <i>+ import permit issued in advance</i>
New Zealand MPI · IHS CATDOG.GEN Import Health Standard veterinary certificate	Status: Not accepted alone Emitter: Private vet Use: Pre-protocol clinical record <i>Official certification mandated by IHS</i>	Model: MPI veterinary certificate Emitter: Official vet (country of origin) Window: ≤ 5 d before export <i>+ pre-issued MPI import permit</i>

Convergent rule — in every analysed jurisdiction, the private veterinary certificate is necessary but not sufficient. The state endorsement (or its functional equivalent under EU and UK schemes, where the emitter must already hold official status) is the document that the border officer accepts. In Peru, the official veterinarian for export is the SENASA-accredited veterinarian.

■ Private veterinary certificate ■ Officially endorsed certificate (border-grade)

World Organisation for Animal Health (WOAH/OIE). Terrestrial Animal Health Code --
Chapter on Certification.

<https://www.OIE/WOAH.org/en/what-we-do/standards/codes-and-manuals/>

Practical tool

Can your pet fly on your exact date?

Calculate in real time: FAVN, microchip, vaccine, quarantine and requirements for 22 countries. Red/amber/green traffic light + reverse timeline + PDF.